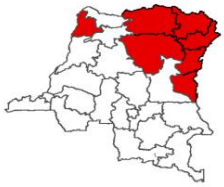
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Ministry of Public Health, Hygiene and Social Welfare
National Institute of Public Health
Public Health Emergency Operations Center
PRESIDENTIAL EBOLA TASK FORCE 17
MVE17 Incident Management System



Situation Report of the 17th Ebola Virus Disease Outbreak / DRC

SitRep N°162/MVE-BDBV/23/10/2026

Reporting date: October 23, 2026 Publication date: October 24, 2026

HIGHLIGHTS (last 24 hours)

Sixty, 60 new confirmed cases and 46 deaths (31 community deaths and 15 within Ebola Treatment Centers) of Ebola Virus Disease (EVD) cases have been reported in the provinces of North Kivu (29), Ituri (28), Haut-Uélé (2) and Tshopo (1). A total of 450 new samples were analyzed; 60 of these samples were found to be positive for the Bundibugyo virus, representing a positivity rate of 13.3%.

No new health zones have been affected in the last 24 hours.

ACCUMULATIVE CASE	ACCUMULATIVE CONFIRMED DEATHS	LETHALITY	PATIENTS IN ISOLATION/CTE	ACCUMULATIVE HEALED	TRACKING RATES CONTACTS (Of the Day)
9,876	4803	48.6%	1018	2,646	88.4%

Affected provinces and affected health zones

8 Provinces affected

Haut-Uélé, Ituri, North Kivu, South Kivu, Tshopo, Bas-Uélé, South Ubangi, and North Ubangi

66 Health Zones affected out of 178 (37.1%)

Summary of key points from the last 24 hours

As of October 23, 2026, **60 new confirmed cases** of Ebola virus disease (EVD) were reported in four provinces: **North Kivu (29 cases), Ituri (28 cases), Haut-Uélé (2 cases) ,** and **Tshopo (1 case).** Among these cases, **31 community deaths** were reported (17 in North Kivu, 13 in Ituri, and 1 in Haut-Uélé). In addition, **15 confirmed deaths** were recorded in Ebola Treatment Centers (9 in Ituri, 4 in North Kivu, and 2 in Haut-Uélé), bringing the total number of confirmed deaths reported during this period to **46 .**

Furthermore, **28 patients (17 in Ituri and 11 in North Kivu)** were declared recovered after obtaining two negative results in the Ebola virus disease (EVD) follow-up test; no recoveries were reported in Haut-Uélé, Tshopo, and Bas-Uélé.

As of October 22, 2026, the DRC had recorded **9,876 confirmed cases, including 4,803 deaths,** representing a case fatality rate of **48.6** %. Ituri province remained the epicenter, accounting for **72.2%** of cases. **%** of reported confirmed cases and **46.7%** of new confirmed cases.

No new health zones have been affected in the last 24 hours.

hours. Thus, 66 out of 178 health zones (37.1%), spread across eight provinces, have been affected since the start of the epidemic.

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Table 1. Distribution of confirmed cases and deaths by affected province, as of October 23, 2026.

Province	New confirmed cases (24h)	Confirmed cases	Deaths (confirmed)	Lethality	Affected health zones
Ituri	28	7135	3,338	46.8%	28/36 (77.8%)
North Kivu	29	2212	1,242	56.1%	17/34 (50.0%)
Haut-Uélé	2	431	189	43.9%	7/13 (53.8%)
Tshopo	1	77	27	35.1%	7/23 (30.4%)
South Kivu	0	3	1	33.3%	1/34 (2.9%)
Bas-Uélé	0	13	4	30.8%	3/11 (27.3%)
South Ubangi	0	2	1	50.0%	1/16 (6.3%)
North Ubangi	0	3	1	33.3%	1/11 (9.1%)
Total	**60**	**9,876**	**4,803**	**48.6%**	**63/167 (37.7%)**

Table 2. Distribution of confirmed cases and deaths by province and zone as of October 23, 2026

Province / Health Zone	Cumulative number			Current situation (24 hours)			
	Case (n)	Deaths (n)	Lethality	New Case	Community deaths (Swab+)	Confirmed deaths within the CTE	Total confirmed deaths
Ituri	**7135**	**3338**	**46.8%**	**28**	**13**	**9**	**22**
Adja	11	1	9.1%				0
Aru	20	6	30.0%				0
Ariwara	8	2	25.0%				0
Aungba	12	5	41.7%				0
Bamboo	216	48	22.2%				0
Boga	2	2	100.0%				0
Bunia	1935	656	33.9%	8	2	3	5
Damascus	41	10	24.4%				0
Drodro	15	7	46.7%				0
Fataki	84	31	36.9%				0
Gethy	11	7	63.6%				0
Kambala	2	0	0.0%				0
Kilo	38	12	31.6%				0
Komanda	192	118	61.5%				0
Lita	317	159	50.2%			1	1
Logo	14	9	64.3%				0
Lolwa	36	9	25.0%				0
Mahagi	5	1	20.0%				0
Mambasa	79	35	44.3%	1	1	0	1
Mandima	402	223	55.5%	15	10	3	13
Mangala	384	168	43.8%				0

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Mongbwalu	717	346	48.3%				0
Nia-Nia	338	197	58.3%	1	0	0	0
Nizi	837	306	36.6%				0
Nyankunde	134	41	30.6%				0
Rimba	21	5	23.8%				0
Rwampara	1179	414	35.1%				0
Tchomia	85	27	31.8%	3	0	2	2
To ventilate	N / A	493	N / A				0
North Kivu	**2212**	**1242**	**56.1%**	**29**	**17**	**4**	**21**
Blessed	570	386	67.7%	10	6	1	7
Good	24	8	33.3%				0
Butembo	326	190	58.3%	3	0	0	0
Goma	1	0	0.0%				0
Kalunguta	87	33	37.9%			1	1
Katwa	656	401	61.1%	3	1	1	2
Kayna	3	2	66.7%				0
Kyondo	44	21	47.7%				0
Lubero	37	14	37.8%				0
Mabalako	93	35	37.6%	5	3	1	4
Alimbongo	5	3	60.0%				0
Manguredjipa	16	5	31.3%				0
Masereka	39	13	33.3%				0
Musienene	156	73	46.8%	2	1	0	1
Mutwanga	37	12	32.4%	2	2	0	2
Oicha	63	30	47.6%	4	4	0	4
Vuhovi	55	16	29.1%				0
Upper Uele	**431**	**189**	**43.9%**	**2**	**1**	**2**	**3**
Boma Mangbetu	45	19	42.2%				0
Dungu	1	1	100.0%				0
Gombari	3	1	33.3%				0
Isiro	109	51	46.8%	1	1	2	3
Pawa	107	61	57.0%				0
Rungu	6	3	50.0%				0
Wamba	160	53	33.1%	1	0	0	0
Tshopo	**77**	**27**	**35.1%**	1	0	0	0
Bafwasende	18	3	16.7%				0
Kabondo	11	2	18.2%	1	0	0	0
Lubunga	1	0	0.0%				0
Makiso - Kisangani	33	14	42.4%				0
Mangobo	10	7	70.0%				0
Tshopo	3	0	0.0%				0

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Wanie-Rukula	1	1	100.0%				0
South Kivu	**3**	**1**	**33.3%**	0	0	0	0
Miti-Murhesa	3	1	33.3%				0
Bas Uélé	**13**	**4**	**30.8%**	0	0	0	0
Buta	1	1	100.0%				0
Viadana	6	1	16.7%				0
Dakwa	1	0	0.0%				0
Ganga	5	2	40.0%				0
South Ubangi	**2**	**1**	**50.0%**	0	0	0	0
Bulu	2	1	50.0%				0
North Ubangi	**3**	**1**	**33.3%**	0	0	0	0
Wapinda	3	1	33.3%				
Total	**9876**	**4803**	**48.6%**	**60**	**31**	**15**	**46**

***These are the confirmed deaths that occurred in the various CTEs in the province which are being broken down to be classified by health zone.**

NA: Not Applicable.

Table 3. Status of alerts notified by province for October 23, 2026

DPS	Alerts Received		Total alerts	Alerts verified and validated (suspected case)		Alerts verified and invalidated (no suspected cases)		Number of suspects investigated	Number of suspects investigated and transferred to CT/CTE (of the day)
	Living	Deceased		Living	Deceased	Living	Deceased		
Haut-Uélé	190	3	193	20	3	170	0	23	5
Bas Uélé	126	7	133	17	7	109	0	24	0
Ituri	1,435	78	1,513	182	76	1,238	2	246	143
North Kivu	2030	44	2,074	194	44	1,210	0	153	86
South Ubangi	ND	ND	ND	ND	ND	ND	ND	ND	ND
North Ubangi	ND	ND	ND	ND	ND	ND	ND	ND	ND
Tshopo	154	1	155	5	1	149	0	3	2
Grand Total	**3,935**	**133 4 068**		**418**	**131**	**2,876**	**2**	**449**	**236**

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Figure 1. Evolution of cases by date of symptom onset over the last 21 days

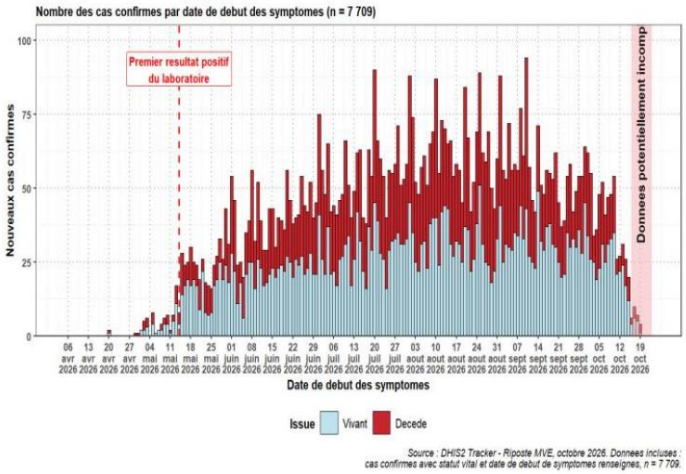
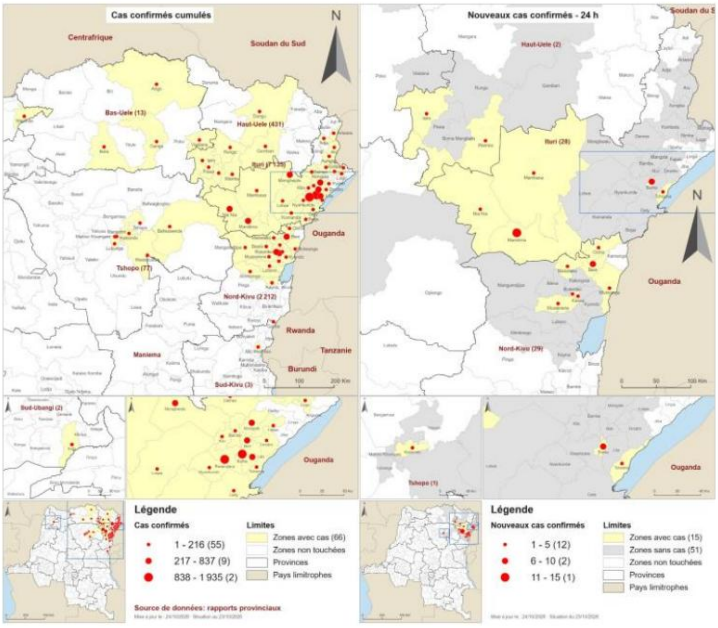


Figure 2. Spatial distribution of confirmed cases (24 hours and cumulative) by health zone.



1. COORDINATION

1.1. Main Actions

ÿ Support for response activities in the affected provinces through the technical pillars.

1.2. RESPONSE ACTIONS BY PILLAR

2.1. Epidemiological surveillance

1.2.1. Main Actions

As of October 23, 2026, **4,068** alerts had been reported in the five provinces that submitted their data, including **3,935** alerts of living individuals and **133** of deaths; **3,427 (84.2%)** were verified, with North Kivu accounting for 626 unverified alerts out of **2,074** (972 of which were reports from the previous day). Of these alerts, **549 (13.5%)** were validated as suspected cases and 2,878 (70.7%) were invalidated. Of the 549 validated suspected cases, **449 (81.8%)** were investigated, with 236 (52.6%) transferred to treatment centers/emergency treatment centers.

ÿ As of October 23, 2026, **36,218** contacts were under follow-up in the five provinces that submitted their data; **32,023** were seen in the last 24 hours, representing a follow-up rate of 88.4%, down from the previous day (89.1%) and below the required threshold of 95.0%. By province, **the follow-up rate was 97.7% in Bas-Uélé (758/776), 93.8% in Ituri (13,450/14,333), 89.2% in Tshopo (1,227/1,376), 85.8% in Haut-Uélé (1,635/1,905) and 83.9% in Nord-Kivu (14,953/17,828).**

1.2.2. Update on checkpoint and point of entry (PoE/PoC) activities

1.2.3. Main actions

ÿ For October 23, 2026, fifteen (15) alerts were notified to the PoE/PoC: 4 in Ituri, 1 in North Kivu, 1 in Tshopo, and 9 in Bas-Uélé. In Ituri, 3 lifeless bodies were intercepted (2 at the Poipo PoC in Mongbwalu and 1 at the Biakato Mayi PoC in Mandima), validated, and swabé, and 1 live alert notified to the DGRPI Nizi PoC was validated and transferred to the Nizi CTE; in North Kivu, 1 live alert notified to the Kahongo PoC (Kyondo) was validated and referred to the Kyavinyonge CSR; in Tshopo, 1 live alert notified to the ONATRA PoC (Makiso-Kisangani) was validated and referred to the Plateau Médical CTE; In Bas-Uélé, the 9 alerts were invalidated after verification.

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Table 4. Status of key PoE/PoC indicators by province, as of October 23, 2026

Indicators	Ituri	North Kivu	South Kivu	Tshopo	Haut-Uélé	Bas-Uélé	Overall
Report completeness	98.4% (61/62)	95% (20/21)	56% (5/9)	100% (18/18)	100% (10/10)	80% (16/20)	93% (130/140)
Strategic PoC/PoE	1	0	0	0	0	0	1
Number of past travelers	165,142	90,327	9,959	21,143	23,340	7,390	317,301
Proportion of travelers screened	98%	98%	99%	99%	99%	98%	98%
% of travelers who washed their hands	98%	98%	98%	99%	99%	99%	98%
% of the travelers sensitized	99%	98%	100%	100%	100%	88%	99%
Number of alerts notified	04	01	0	01	0	09	15
Number of live alerts validated	01	01	0	01	0	0	3
Number of lifeless bodies intercepted	03	00	0	0	0	0	3
% of suspected cases transferred to CT/CTE	100% (1/1)	100%	0%	100%	0%	0%	100% (3/3)
% of lifeless bodies swabed/secured	100%	0%	0%	0%	0%	0%	100% (3/3)
% of suspected cases investigated within 24 hours	100% (4/4)	100%	0%	100%	0%	0%	100% (6/6)

1.2.4. Challenges

ÿ **In North Kivu,** the Butuhe PoC remains non-functional since the security incident of 26/09/2026 and the alert from the Kahongo PoC was referred to the Kyavinyonge CSR and not to a CT/CTE; in South Kivu, the completeness of reports remains low (56%; 5/9) and the Kavimvira, Kalundu Port SNCC and Baraka PoEs do not have isolation devices; in Bas-Uélé, 4 out of 20 PoCs did not report (80%; 16/20) and only 88% of travelers were sensitized; in Ituri, the Avakubi PoC is not functional due to insecurity and nearly 30% of travelers (969/3,247) refused screening at the Pluto PoC (Mongbwalu).

1.3. Laboratory

1.3.1. Main actions

ÿ **Ituri :** 28 new positive results (15 living and 13 dead) out of 225 new samples analyzed (positivity: 12.4%).

ÿ **North Kivu :** 29 new **positive results (12 living and 17 dead) out of 179 new samples analyzed in Beni and Butembo (positivity: 16.2%).**

ÿ **Haut-Uélé: 2 new positive results (1 living and 1 dead) out of 5 new samples analyzed (positivity: 20%).**

ÿ **Tshopo: 1 new positive result (1 living and 0 deaths) out of 13 new samples analyzed (positivity: 7.7%).**

ÿ **Bas Uélé: 13 samples received and analyzed (13 live) the results are revealed to be negative.**

1.3.2. Infection Prevention and Control (IPC/EDS)

1.3.3. Main actions

ÿ **Ituri :** 24 out of 29 rings were opened (82.8%), 47 out of 54 households were decontaminated (87.0%), and 7 out of 7 health facilities were decontaminated (100.0%). Of 71 EDS alerts (including 11 postponements), 47 EDS were carried out (66.2%) and 47 bodies were swabed.

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- **North Kivu :** 29 out of 51 rings were opened (56.9%), 21 out of 26 health centers were decontaminated (80.8%), and 11 out of 33 households (33.3%); 9 infection prevention and control (IPC)/water and sanitation (WASH) assessments were conducted in 9 health centers (average score of 33.5%), 26 schools in Katwa received IPC/WASH kits, and 14 schools in Butembo were decontaminated. Of 64 death and health alerts (DHHA), 57 DHHAs were carried out, and 60 bodies were swabbed.
- **Haut-Uélé :** 9 out of 9 households decontaminated (100.0%) in Wamba and 2 out of 2 ESS (100.0%) in Isiro and Wamba; 42 service providers briefed (22 in Wamba, 17 in Isiro, and 3 in Pawa), as well as 25 members of the IPC team from Boma Mangbetu; 3 ESS assessed in Isiro (Arindru Health Center, Nduka Health Center 33.7%, and Nolua Health Center 37%); and 5 triage units operational in Wamba. Of 12 EDS alerts, 10 EDS were carried out (83.3%) and 10 bodies were swabed.
- **Tshopo :** 3 rings were opened in Makiso-Kisangani; Alwaleed Hospital (case no. 77), Kongakonga CSR and Makiso-Kisangani HGR were decontaminated, 2 ESS were provided with PCI kits, 9 out of 9 ESS were monitored in 5 ZS and 121 out of 121 providers were briefed; Bafwasende HGR was assessed (37.1%) and Kondima and Bombula CS were reassessed (80.6% and 67.7%).
- **Bas-Uélé :** 11 out of 11 ESSs were followed up in 3 ZS (Buta, Viadana and Ganga), 46 out of 46 providers and 16 out of 16 hygienists were briefed and 3 assessments were carried out in the ZS of Buta (CH Rubi Lisanga 54%, CSR Mobenge 72% and CSR Triangle 47%); out of 7 EDS alerts.

1.3.4. Challenges.

- **In Tshopo,** 7 out of 107 health establishments (6.5%) exceed 80% on the IPC score, 1 body could not be swabed, and the investigation into case #54 remains blocked by household resistance. 17 out of 20 (85.0%) lack water, and only 130 out of 632 service providers (20.6%) are trained in IPC-WASH. In Haut-Uélé, only 4 out of 7 health zones reported cases. 5 confirmed cases from the Bolebole axis remain in the community, preventing the decontamination of their households. IPC kits are lacking in health establishments, and IPC supervisors are not activated in 4 health zones (Rungu, Dungu, Boma Mangbetu, and Gombari).

1.3.5. Holistic care

1.3.6. Main actions

- **In Ituri,** 93 new admissions were recorded compared to 99 discharges, including 17 recoveries, Thus, 397 patients are hospitalized, representing an occupancy rate of 38%; 368 out of 379 patients (97.1%) received hot meals and all children under LNPE/SLM were taken care of (18/18 at the CTE, 5/5 at the nursery and 54/54 in the community).

- **In North Kivu,** 95 new admissions were recorded compared to 125 discharges, including 11 recoveries. Thus, 447 patients are being treated, including 61 children under 5 years old, 5 critically ill patients and 107 outside of CTE/CT, for an overall occupancy rate of CTE/CT of 82.9% (340/410); 23 ambulance trips were successfully completed (8 out of 14 ambulances used).

- **In Haut-Uélé,** 15 new admissions were recorded against 6 discharges; 41 patients are hospitalized for 158 beds, representing an occupancy rate of 25.9%, and all benefited from hot meals, as did 68 accompanying persons and 91 PPLs.

- **In Tshopo,** 4 new admissions (1 confirmed PPL and 3 suspected) were recorded against 2 discharges (non-cases); 15 patients are hospitalized for 36 beds, representing an occupancy rate of 41.7%, and all received hot meals, as did 3 accompanying persons and 47 PPLs.

- **In Bas-Uélé,** 17 new admissions of suspected cases were recorded and isolated in the ESS without any discharges, bringing the number of patients in isolation to 118; the 3 beds for confirmed cases are occupied (100.0%).

1.3.7. Challenges

- **In Tshopo,** the contamination of PPL at the CTE (admission of a confirmed nurse from the CTE Météo) requires reviewing the clinical PCI and 4 confirmed cases still refuse hospitalization (3 in Bafwasende and 1 in the Tshopo health zone).

- **In Bas-Uélé,** none of the suspected patients isolated in the ESS benefit from nutritional support, one isolated patient escaped due to lack of food care and the standard CTE remains under construction.

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ÿ **In Haut-Uélé,** a confirmed case escaped from the Boma Mangbetu Ebola Treatment Center (ETC), the Pawa ambulance remains immobilized for maintenance, network coverage remains weak in some health zones and some staff are demotivated; in Ituri, blood remains insufficient, 11 patients in Mandima (9 at the Mandima Treatment Center and 2 at the Makeke Treatment Center) have not received food, ambulance movement reporting remains low, the temporary ETC in Biakato May lacks drinking water and, at the Biakato Mines ETC, bodies are decontaminated in the same room as living patients; in North Kivu, the beds for suspected cases are occupied at 91.7% (176/192), 107 out of 447 patients (23.9%) are being treated outside of ETCs/Treatment Centers and only 8 out of 14 ambulances are being used.

1.3.8. Vaccination 1.3.9.

Main actions

In Ituri, vaccination of people with low blood protein (LBP) using Ervebo and research on its efficacy against BDBV continue, bringing the cumulative total to 1,613 vaccinated individuals (963 men and 650 women, including 21 pregnant women and 39 breastfeeding women), with 31 reported adverse events following immunization (AEFIs) (26 non-severe and 5 severe). Clinical trials for the TUMAINI project have been officially launched. In Bas-Uélé, the cumulative total has reached 1,660 vaccinated individuals out of a target population of 13,202 (12.6%), primarily in Buta (648; 22.6%) and Ganga (273; 20.5%). Compassionate vaccination continues in the health zones of Ango and Buta, and pre-registration of LBP and TBP is underway. In Haut-Uélé, LBP vaccination remains suspended. Data from North Kivu and Tshopo are not yet available.

1.3.10. Challenges

ÿ In Bas-Uélé, the start-up of the UCCs is not effective and vaccines as well as vaccination funds remain insufficient; in Haut-Uélé, the breakdown of the generator at the PEV antenna in Isiro prevents the start-up of the 2 UCCs and the vaccination of PPLs; in Ituri, the target of TPLs and PPLs remains to be redefined and 5 serious AFIs have been notified.

1.3.11. Continuity of Care

1.3.12. Main actions ÿ **In North Kivu,**

the baseline assessment of the CSSE at the Katwa General Referral Hospital reveals the absence of a simplified CSSE plan, triage not respecting the circuit, a shortage of PPE and blood products, as well as the absence of an ambulance and a contingency plan in case of absenteeism; 18 out of 178 staff are absent and 3 are in quarantine.

ÿ **In Haut-Uélé,** the workshop for engaging non-traditional partners in vaccination started in Isiro (October 23-24), tokens are being deployed to collect zero-dose and under-vaccinated children during the distribution of LLINs scheduled for October 27, the confirmed PPL in Wamba is being monitored and the vaccination session at the Gosamu Health Center was supervised; in Wamba, Penta3 coverage fell from 83.8% in Q2 to 23.9% in Q3 after the closure of 4 ESS.

ÿ **In Ituri,** medicines intended for free care have been deployed in the ESS of Bunia and Rwampara, deployment is underway in Komanda and Nyankunde and the batches from Mambasa and Nizi have arrived in Bunia; 9 out of 25 expected to recover at the first follow-up and 21 at the second have benefited from medical and biological follow-up (100%) and the rehabilitation of the second clinic for recoveries in Bunia continues.

ÿ **In Tshopo,** the Bombula, Pharmat and St Francis of Assisi health centers were evaluated: the triage area is not effective in any of the visited health centers, the partogram is not used for monitoring deliveries (Bombula and Pharmat), 12 children missed their CPS appointment (8 at Pharmat and 4 at Bombula) and 14 out of 16 staff (87.5%) are vaccinated at St Francis of Assisi; the recovery plan for the Marie Antoinette health center was developed.

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1.3.13. Challenges

ÿ North Kivu, CSSE plan and absenteeism contingency plan to be developed at Katwa General Hospital.

In Ituri, only 472 out of 717 recovered patients (65.8%) are being followed up, the Bunia clinic for recovered patients is overused and some medications and supplies are out of stock; implementation of recovery plans for assessed health services still to be ensured and incinerators not operational (St. Francis of Assisi and Pharmat) in Tshopo, where the use of IMCI and maternal health services (deliveries and CPON3) continues to decline; initial assessments of the CSSE are not effective in the Lubunga health zone and the EPI continuity plan needs to be activated; in Haut-Uélé, vaccination of people with low birth weight is stopped, the catch-up vaccination project (UIV) is delayed, there is a shortfall in subsidies for free care in the 7 most affected health zones and follow-up of chronic patients is interrupted in Wamba, where the tuberculosis cure rate fell to 0% in Q3.

1.4. Risk Communication and Community Engagement (RCCE)

1.4.1. Main actions

ÿ **In Ituri,** 8,630 home visits reached 44,130 people, 562 educational talks reached 11,486 people, 26 mass awareness campaigns reached 2,821 people and 8 community debates reached 436 people; 571 community alerts (including 128 deaths) were notified, 323 feedbacks out of 365 (88.5%) and 7 out of 9 incidents (77.8%) were resolved within 48 hours, the pre-listing of 409 contacts around 11 confirmed cases and the transfer of 47 out of 50 suspects to the CTE were facilitated, but only 12 out of 28 health zones (42.9%) reported. In North Kivu, 15,309 home visits reached 51,159 people, 295 educational talks reached 5,814 people and 166 mass communications reached 8,843 people; 445 community alerts were notified out of 650 expected (68.5%), 23 feedbacks out of 31 (74.2%) and 1 incident out of 8 (12.5%) were resolved within 48 hours and 2,186 people out of 4,750 (46.0%) were engaged in community actions.

ÿ **In Haut-Uélé,** 115 home visits reached 635 people, 39 educational talks reached 805 people and 4 mass awareness campaigns in schools reached 4,578 people; 11 community alerts were notified (6 in Isiro and 5 in Wamba) and 25 out of 25 feedbacks were resolved within 48 hours; village committees helped identify 5 patients hidden in the Bakanja health center (Wamba), who were transferred to the Wamba treatment center, and the integrated training of 30 community health workers was completed in Isiro.

ÿ **In Tshopo,** 7 mass awareness campaigns reached 8,300 people and 2 educational talks reached 149 people, 4 radio programs were hosted and 56 posters and 108 advice cards were distributed; 2 out of 2 feedback issues were resolved within 48 hours, 2 out of 2 refusals were lifted and 2 incidents were recorded (attack on teams at PK 15 and fire at the house of a RECO in Bafwasende), which led to advocacy with the FARDC in Bafwasende.

ÿ **In Bas-Uélé,** 110 home visits reached 768 people, 2 educational talks reached 32 people and 3 mass awareness campaigns reached 669 people and 1 radio program was hosted in Buta; 8 out of 8 community alerts were notified and resolved (Likati and Ganga) and awareness campaigns in schools reached 493 students and teachers in Likati and Poko.

1.4.2. Mental Health and Psychosocial Support (MHPSS)

1.4.3. Main actions

ÿ **In Ituri,** 14 new confirmed cases out of 27 (51.9%) benefited from SMSPS interventions in 15 affected health zones covered out of 28 (53.6%), 15 households of separated and orphaned children

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out of 17 (88.2%) were accompanied, 78 children are being monitored, 9 out of 9 children are in daycare and 12 who have recovered have been reintegrated.

ÿ **In North Kivu,** 24 new confirmed cases out of 28 (85.7%) and 196 confirmed cases under follow-up benefited from SMSPS interventions, 188 new suspects and 245 suspects under follow-up were taken care of, 37 households of separated and orphaned children out of 40 (92.5%) were accompanied, 28 out of 36 children (77.8%) followed in the community, 6 out of 6 children in nursery, 8 recovered reintegrated and 93 results announced, including 27 positive; 517 people benefited from psychoeducation.

ÿ **In Haut-Uélé,** 2 confirmed cases, 4 confirmed cases under follow-up, 2 new suspects and 15 suspects under follow-up were taken care of, 7 results were announced, including 2 positive ones, 4 separated children are being followed in the community, 18 caregivers and 47 PPLs were supported and 3 EDS supported; data from Wamba were not received.

ÿ **In Tshopo,** the new confirmed case (1/1; 100.0%) and the 4 confirmed cases under follow-up benefited from SMSPS interventions, 2 out of 2 new suspects and 8 suspects under follow-up were taken care of, 3 out of 3 resistances were lifted (including the isolation and sampling of 3 suspects in Umoja, Kabondo), 3 PPLs were supported and 106 people, including 51 military personnel in Bafwasende, benefited from psychoeducation.

1.4.4. Challenges

ÿ **In Haut-Uélé,** no crèche is available for separated children (Isiro, Pawa, Wamba and Boma Mangbetu), the mobility of APS is lacking in Wamba and Boma Mangbetu and nutritional kits for weaned infants are insufficient; in Ituri, the coverage of interventions around cases remains insufficient, particularly in Mandima; in North Kivu, SMSPS coverage remains weak around the latest confirmed cases in 9 ZS and 10 affected ZS do not have a crèche.

ÿ **In Bas-Uélé,** the psychosocial support for the 3 confirmed cases is still lagging behind; in Tshopo, the APS of the Wanie-Rukula health zone still need to be trained and the pillar lacks communication units and means of transport.

1.4.5. Logistics
1.4.6. Main actions

ÿ **In Tshopo,** the construction of the Bafwasende CTE, the extension of the standardized Météo CTE, the repair of the incinerator (successful test at over 1,000 °C) and the maintenance of the laboratory air conditioning are continuing, the Bafwasende sub-coordination has been supplied with inputs and preparations for the mid-term review are underway, but 115 motorcycles are still missing from the ZS; in Bas-Uélé, the SGI has been supplied with 200 liters of diesel and the construction of the HGR Buta CTE (partitioning and toilets) as well as the rehabilitation of the DPS offices are continuing, but ambulances, computer equipment and team mobility are lacking; In North Kivu, 3 UNICEF tents were received, one tent was delivered to the CTE of the Beni General Referral Hospital and inputs for IPC, EDS and care were delivered to the health zones of Butembo, Katwa and Oicha.

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1.4.7. Security

1.4.8. Main actions

ÿ **In North Kivu,** the attack on response teams at PK 15 (Buta road) and the burning of a RECO's house in Bafwasende were the subject of a plea to the FARDC; in Ituri, the Avakubi PoC remains non-functional due to insecurity and, in North Kivu, the burials of 4 confirmed cases (Bundji and Musienene) as well as the decontamination of 2 ESS (AS Mangote) and a household (AS Kasanga) were secured, while the Butuhe PoC remains closed since the death of a service provider on 26/09/2026.

1.4.9. PSEA (Prevention of Sexual Exploitation and Abuse)

1.4.10. Main actions

In Tshopo, 60 participants were sensitized on the PEAHS (Public Health and Social Care Program), the referral process, and zero tolerance. In Haut-Uélé, 24 healthcare providers from the Pimpester Sister Rose Clinic (3 men and 21 women) and 27 teachers from Saint Vincent de Paul College (18 men and 9 women) were sensitized on the PSEAH (Public Health and Social Care Program) in Isiro, and 27 codes of conduct were signed. In Ituri, 15 hygiene workers from the Bunia CTE (Emergency Treatment Center) (9 women and 6 men) were briefed on the PSEAH and signed the code of conduct, and 55 traditional healers (35 women and 20 men) were sensitized in Tchomia. In North Kivu, 10 focal points from the Beni community health workers (9 women and 1 man) were briefed and signed the code of conduct, and 143 people were sensitized in Manguredjipa, Butembo, Biena, and Musienene. Data from Bas-Uélé was not received.

1.4.11. Challenges

ÿ Coverage of affected areas remains low: in Tshopo, data collection tools are lacking and the CBCM (Community-Based Monitoring Center) has yet to be installed in 3 health zones in Kisangani; in Ituri, only 15 out of 36 health zones (41.7%) are aware of the reporting mechanism and only one partner (UNICEF) reports its activities; in North Kivu, only 6 out of 34 health zones (17.6%) are aware of the reporting mechanism; in Haut-Uélé, only 6 out of 13 health zones (46.2%) are aware of the reporting mechanism, none of the 14 partners report their PSEA (Public Health and Social Action) activities, and awareness-raising tools are only available in Swahili; at the national level, only 12 out of 56 health zones (21.4%) conducted PSEA activities during the week of October 16-22, 2026, and only 4 out of 15 partners supported these activities.

KEY MESSAGES

Over the past 24 hours, 449 out of 549 suspected cases (81.8%) have been investigated in the five provinces that submitted their data; 100 remain uninvestigated (85 in North Kivu, 12 in Ituri, and 3 in Tshopo), and none of the 24 confirmed suspects in Bas-Uélé have been transferred to a treatment center/emergency care facility. The contact tracing rate has fallen to 88.1% (31,923 out of 36,218), compared to 89.1% the previous day, and remains below the required threshold of 95.0%, with a critical situation in Nia-Nia (66.0%), Wamba (69.7%), Lita (70.7%), Beni (71.3%), and Kalunguta (71.4%). In North Kivu, 626 out of 2,074 alerts have not been verified. This situation requires a strengthening of the investigation of suspects and the monitoring of contacts in order to limit community risks.

of transmission

Maintain a level of vigilance regarding epidemiological surveillance aspects, consolidate actions.

TIMELINE OF KEY EVENTS

From April 24 to May 15, 2026, the epidemic progressed from the detection of the first suspected cases to the biological confirmation of the Ebola Bundibugyo virus, leading to the official declaration of the 17th Ebola Virus Disease epidemic in the Democratic Republic of Congo.

Between May 17 and 18, 2026, national authorities, with the support of partners, strengthened the response by declaring a public health emergency of national and then continental scope, accompanied by the mobilization of funds.

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